

Dissection was widely performed in the nineteenth century. There was a shortage of bodies to be dissected, as the number of medical schools kept rising, while the supply was constant and up until 1832 it officially came only from the bodies of murderers. This caused several bills to be passed, e.g. the 1832 Anatomy Act, each of which increased the supply. The 'Paris model', i.e. a model of teaching medical students based on practice, gained traction and doctors saw an increased need to perform dissection both to train and to gain anatomical knowledge. Due to the shortage, the doctors paid the so-called body snatchers to dig up bodies from the graves, and some people murdered others to sell their bodies. The targets were usually marginalized people of low socio-economic status, and in the US it was the African Americans. The 3 main reasons for the controversy surrounding dissection are the ways of obtaining the bodies, people's beliefs and associations regarding dissection, and the exploitation of vulnerable groups.

From 1540 on, the Company of Barbers and Surgeons was entitled to the bodies of 4 hanged criminals each year. The 'Murder Act' passed in 1752 said that judges could add dissection to hanging when sentencing. In 1832 the Anatomy Act was passed. According to it, workhouse masters, hospital managers, and Poor Law guardians could send unclaimed bodies to anatomists. Inspectors were introduced.

People's bodies were given away for dissection if no one came to claim them within 48 hours. Sometimes the bodies were secretly dissected even after being claimed or before the said 48 hours. The body of workhouse inmates were dissected in this way. Their relative came to claim their bodies, and in the funeral hearse there was someone else's body. This caused a major controversy in 1857. St Mary Newington workhouse master, Alfred Feist, was accused in the Central Criminal Court of unlawfully disposing of the bodies and giving them to Guy's Hospital medical school. The family was not informed of the death of a patient to make it harder to claim the body. The patients had to be constantly visited to be sure that they wouldn't be dissected.

Patients were also dissected in asylums, e.g. in the West Riding Pauper Lunatic Asylum. The family was informed but preventing the dissection was made hard. "In case of death the usual postmortem examination will be made in order to certify correctly the cause of death. Relatives in any case objecting to this course are requested to communicate immediately upon receipt of this notice, personally, with the Medical Superintendent." - read a part of the admission letter sent to relatives of patients in the West Riding Asylum.

Body snatchers robbed corpses from the graves. They were paid for the corpses. From 1832 on, they were supposed to present a certificate stating that the body was obtained legally, but usually that didn't happen. Sometimes, doctors unknowingly accepted even body parts, which came from murder due to the lacking certificate. The inspectorate did not act. Bodies had to be extracted within 10 days of burial in order to be fresh. Many

countermeasures were devised, e.g. iron cages, iron coffins, storing the corpses to let them decay before burying them, or guards. Guards were the most common and the least expensive countermeasure but they could be bribed with whiskey or money. Sometimes family members stood guard.

People murdered others just to sell their bodies. In 1886 a porter from the University of Maryland School of Medicine strangled Emily Brown and sold her body to the university for \$15.00. In 1828 two Irishmen living in Edinburgh, Burke and Hare murdered 16 people to sell their bodies to a private anatomy school of Dr Robert Knox.

The inspectorate didn't enforce the law and instead supported body snatching. It followed the precedent set by the first called inspector, James Somerville, who was appointed in 1832. Inspectors had too little power to prevent grave robbing. Somerville reported that his job was "one continued struggle ... to enforce the regulations". There was too much work and too few inspectors, so they relayed the work to undertakers, who did as they wished, e.g. John Dix. They, for example, delivered the bodies without the certificate, which was one of the act's "most stringent requirements". The inspectors had to make the medical schools happy by providing supply and keep the public misinformed to prevent a scandal. Supply kept rising due to many factors. First of all, the 'Paris model' emphasized dissection. What's more, the 1815 Apothecaries Act said that GPs should have some anatomy training. According to the 1858 Medical Registration Act, doctors had to study anatomy before getting a medical licence.

Doctors often dissected fetuses, some of which came from infanticide. An unborn child is innocent and it may be inappropriate to dissect it. After the law, which made single mothers receive neither state support nor money from the fathers, many women resorted to infanticide. Fetuses were preserved and kept in museums and personal collections due to their small size.

People believed that after dissection one could not go to heaven. Thus they didn't see a point in burying someone after dissection. People were not buried properly.

Dissection was associated with criminals, on whom, under the 1751 Murder Act, it was imposed as a punishment only for murder. Before the Anatomy Act was passed, bodies could be only legally obtained from the gallows.

Doctors were believed to have intercourse with female bodies. An example of that is the case of Mary Paterson. She was a young prostitute and was depicted by an artist in a sexual pose on the dissecting table. Additionally, a porter at St Bartholomew's was said to have raped the corpse of an attractive fifteen-year-old girl in front of medical students.

Galvanic experiments were conducted on the corpses. Professor Giovanni Aldini came to London to perform them. They did not work. Even though the public enjoyed them, it was as if the doctors played at being God. Also, it would be problematic from a legal

perspective if a murderer was brought back to life. There was social anxiety about the boundary between life and death.

Nineteenth-century medical students were thought of as rowdy, ill-behaved and indulging in heavy drinking. People might have been scared to let such people handle them after death. Some of them, for example, performed pranks using body parts. Once a student in London threw an amputated leg from a roof down a chimney, which caused a riot.

In the US it was the black people whose graves were robbed. Black people had neither the power nor the money to go after body snatchers. In the south of the US, white people spread fake stories of “night doctors” roaming the streets of cities in the north, waiting to murder black people and dissect them. In 1989 construction workers working in the building of the Medical College of Georgia found around 9000 human bones, 80% of which were from African Americans. The college’s building has been in use since 1835.

One source of corpses for dissection were the unborn children of poor single mothers that committed infanticide. In 1877 Florea McLean that wanted to see her dead baby saw it severed and stitched back with its head missing. It only added to the grief of a miscarriage. Some mothers felt elation that their child died and wanted to get rid of it as soon as possible.

Poor people from workhouses or hospitals, whose relatives didn’t hold guard were at risk of being dissected. Dissection saved the workhouses burial costs. Also, some hospitals had a deal with them, in which they provided free healthcare to the inhabitants of a workhouse.

Prisoners’ bodies were also used. Between 1832 and 1844 St Bartholomew’s hospital obtained bodies from prisons and workhouses for £5 each.

Practice of dissection was controversial. Currently, less so. People donate their bodies for dissection voluntarily. In many hospitals it is the Caucasians who are the main source of the bodies. For example, in the years 2003-2006 out of the 203 bodies received by the Duke University School of Medicine, 97% were Caucasian. While dissecting and preparing human bodies for medical purposes is not so controversial nowadays, doing it for entertainment, as in the case of the Body Worlds - a travelling exposition of human bodies, does raise some people’s concerns. The ways of obtaining the bodies, people’s beliefs and attitudes, and the exploitation of marginalized groups were the main factors in creating controversy among dissection in the nineteenth century. Nowadays, all of them have changed - bodies are obtained from donors, people no longer associate dissection with criminals, and vulnerable groups are no longer the main supplier of bodies.

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